

Incident Cards: A Tool for Documenting and Reviewing Experiences with Chronic Conditions

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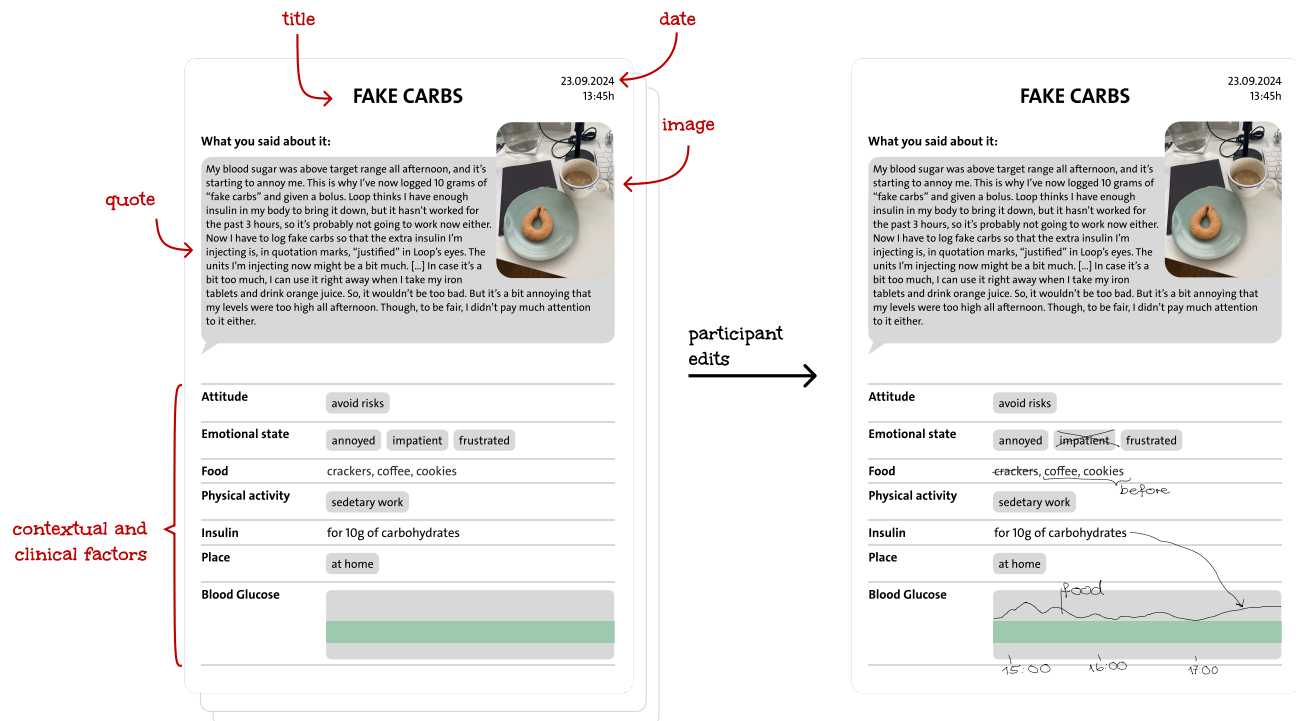


Figure 1: (left) Incident Card of a participant featuring a title, date, participant quote, participant image, and contextual and clinical factors. (right) the same Incident Card with edits added by the participant during the workshop.

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Abstract

For researchers, capturing the lived experience of individuals with chronic conditions poses significant challenges; however, this is crucial for effective design and research in these settings. We propose Incident Cards, an artifact designed to help researchers and designers translate these experiences into a structured format that

facilitates documentation, recollection, review, and reflection. Inspired by the Critical Incident Technique, Incident Cards offer a comprehensive and holistic view of self-management experiences, capturing both clinical and contextual factors. Our method for creating Incident Cards includes guidance on data collection and analysis, to ensure relevance and usability across diverse contexts. We present an application of Incident Cards on a diary study and follow-up workshop of 10 individuals living with type 1 diabetes. We discuss Incident Cards' value and practical application in interviews, workshops, and generative design activities. Our findings indicate Incident Cards have potential to help researchers ground experiences, foster engagement, and support reflection.

CCS Concepts

• **Human-centered computing** → **Human computer interaction (HCI)**; **Interaction design**.

Keywords

Critical Incidents, Reflection, T1D, Health, Chronic Conditions, Design Artifact

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1 Introduction

Chronic conditions such as diabetes, heart disease, stroke, and cancer are the leading causes of morbidity and mortality worldwide [37, 86]. With an aging population, the prevalence of chronic conditions increases [37] and contributes to preventable deaths, disabilities, and health care costs [85]. Capturing participant's experiences of chronic condition management and translating experiences into actionable insights for research and design is challenging for researchers. Challenges include documentation of self-management practices enriched by contextual factors, recall of experiences minimizing cognitive or retrospective bias, and reflection on experiences in collaborative settings. To address these challenges we propose INCIDENT CARDS, depicted in Figure 1, research artifacts designed to support researchers and designers in documentation, recollection, review, and reflection of chronic conditions management practices. INCIDENT CARDS translate individual experiences into artifacts, encapsulating people's experiences through direct quotes, images, and contextual and clinical factors. They can be used by researchers in qualitative interviews, workshops, brainstorming, and other participatory design activities. The name "incident card" is inspired by the critical incident technique, focusing on lived experiences and relevant contextual factors influencing an outcome [11, 12], following Butterfield et al. [11]'s later expansion and literature showing the importance of context in chronic conditions [57, 71]. We consider *an incident* a concrete example of an event, experience (positive or negative), or behavior that is relevant to the person experiencing it and contributes to the understanding of a situation. We also present the creation of INCIDENT CARDS, including

data collection and analysis as a method to guide researchers in creating their own INCIDENT CARDS. We report on our experience of employing INCIDENT CARDS and how they shaped the process and outcomes of a diary study and workshop with 10 individuals with type 1 diabetes (T1D) as a use case. Our contributions are threefold. First, we present INCIDENT CARDS, a research artifact to support qualitative research and generative design by helping participants externalize their experiences in a structured format. Second, we provide guidance on how researchers can create and use the cards in different contexts considering data collection, domains, and research goals. Third, we present our insights and reflections on employing INCIDENT CARDS in a workshop, including grounding participant discussion, contextualizing and externalizing; supporting reflection, and facilitating engagement.

2 Related Work

2.1 Techniques for Participant Experience Capture and Analysis

Several techniques in HCI aid in the process of documentation, recollection, and reflection on experiences in individual and group settings. For example, diary studies and experience sampling method have been designed to collect data about participants' daily activities, thoughts, and feelings [6]. Flanagan [28] proposed the critical incident technique (CIT), a set of procedures to observe human behavior. A *critical incident* describes an observable human behavior with a clear purpose, and consequences [28]. CIT has been adopted in qualitative research in different fields [83] and integrated into various methods [83] such as face-to-face interviews [11]. The focus of CIT broadened to identifying critical factors, events, behavior, or experiences that influence activity [11, 12]. Focusing on experiences with a product, space, or system Buchenau and Suri [10] proposed experience prototyping to enable understanding, exploring, and communicating experiences. In education studies, stimulated recall interviews support recollection during or after learning activities [87]. INCIDENT CARDS build upon this work by providing an artifact that succinctly encapsulates participant experiences to support in discussion or design activities.

2.2 Engaging Participants in Reflection and Contextualization

Encouraging participants to reflect and contextualize their experiences is key to gaining insights into living with chronic conditions. Techniques to support reflection include capturing and visualizing data such as images, location, user-defined tags [67, 80], or using Lego to visualize blood pressure [4]. HCI research has explored self-reflection to improve health [27], leverage contextual factors [17, 71], support self-development, enable critical review [30, 63], and inform design in personal informatics [22, 24, 50]. Techniques exist to facilitate participants' reflection in collaborative studies, ensuring grounded discussions and rich insights, such as collaborative reflection [70]. Collaborative reflection helps individuals exchange experiences, learn from each other [20, 70], and generate insights that can only emerge through collaboration [44, 62, 69, 77]. Other research includes images and whiteboards

to trigger reflection and experience exchange [29, 30, 46], or reflective prompts to guide discussion [65]. Beyond these techniques, design toolkits, particularly design cards [42], provide structured ways to support design activities that rely on reflection, discussion, and creative exploration. Design cards have been used to support ideation [32, 38, 54], co-design [55], formative evaluation of design concepts [21], and advocating design decisions to stakeholders [18]. Application domains including IoT applications [64], data protection [56], sharing economy [25], sustainability and design justice [26], and behavior change [19]. While design cards often focus on generative design activities and support divergent steps of the design process, e.g., help structure ideation, design and prototyping phases, INCIDENT CARDS primarily attend to capturing lived experiences, providing structured artifacts for research and (self-)reflection.

Helping participants contextualize experiences to communicate a holistic picture is challenging. Without sufficient contextual data communicated appropriately, patients' critical thinking is limited [75], data interpretation and value are challenged [88], and biases, such as confirmation bias, might be reinforced [58]. To model the relationship between context and self-management practices, Raj et al. [72] introduced contextual frames to support the design of context-aware systems. INCIDENT CARDS complement this work by supporting participants to externalize relevant contextual factors and use them to inform design.

3 Creation of Incident Cards

We illustrate how to create INCIDENT CARDS using established HCI methods to generate artifacts that facilitate discussions and design activities.

3.1 Data Collection

To ground incidents in participants' experiences, researchers can use methods in which participants self-collect data, such as feedback or elicitation diary study [15] or experience sampling methods [39]. Alternatively, researchers can use approaches in which participants recall concrete incidents of interest, such as the CIT [28], or research takes place in the user's natural environment, such as contextual inquiry [40]. The focus of data collection should be descriptions of incidents that participants experienced.

Images aid experience recall and reflection [2, 41, 73], so INCIDENT CARDS include them. Images can be captured by participants, e.g., in a diary study, or by researchers, e.g., during observations. In cases where image capture is not possible, researchers could create a representative illustration in collaboration with participants.

If relevant contextual and clinical factors can be predefined (see Section 3.3) it is worth considering if the data collection strategy yields data that can be translated into the appropriate structure. However, data collection should prioritize capturing experiences rather than exhaustively covering all factors to avoid compromising the authenticity of experiences.

3.2 Data Analysis: Selection of Incidents

Researchers then select which incidents to include to support the activities for which the INCIDENT CARDS are to be used. Data analysis is an iterative and non-linear process, and steps relevant to incident selection are detailed below.

Data familiarization. Researchers may adopt approaches such as the familiarization phase of the reflexive thematic analysis of Braun and Clarke [9] or other methods to immerse themselves in the collected data [40, 43, 48].

Requirements definition. The requirements formulated and explicitly written down by researchers can range from concrete to vague. An example of a concrete requirement could be to only include incidents about participants' experience with physical activity, while a more vague requirement could be any interesting participant behavior related to self-management practices.

Mapping. Once requirements have been defined, incidents should be mapped to requirements. Revisiting and discussing the identified incidents collaboratively can be beneficial to ensure alignment and comprehensive coverage of the requirements. This step will result in a set of initial incidents.

Participants' validation. When defining the final number of incidents to select for INCIDENT CARD creation, researchers should keep the number of incidents proportional to the planned activity and allotted time. INCIDENT CARDS may contain sensitive information, so researchers should take care that incidents are appropriate to the planned activity and prioritize participant comfort [49, 51].

Determine final incident selection. To determine the final set of incidents, researchers should consider alignment with their research question, context (design activities, interview, group discussion), and goal of using the INCIDENT CARDS. For example, when using INCIDENT CARDS for design, identifying common themes can be beneficial, as shown in practice-based design inquiry [34, 79], usability [47] and user experience evaluation [60], and eHealth service design [13]. If the context of use is an interview, researchers may prioritize incidents they find most insightful for deeper reflection. Participant background and incident diversity can affect research outcomes. For example, if participants have diverse backgrounds and incidents are also diverse, discussion may introduce multiple perspectives, making it difficult to establish common ground.

3.3 Combining Clinical and Contextual Information for Incident Card Creation

To understand the contextual and clinical factors relevant to a researcher's use case and research goal, several frameworks exist. Bardram et al. [3] discussed contextual triggers for context-aware systems for affective disorders, differentiating physical, behavioral, social, cognitive, and disease-related factors. Ghahari et al. [35] categorized context-related factors for multiple sclerosis management into physical environments, social networks, and community. Raj et al. [72], proposed contextual frames to model the context behavior relationship in individuals with T1D. With the rising prevalence of multimorbidities [1, 7, 31, 74, 78, 84], combining contextual and clinical factors of multiple conditions can be valuable. Such established definitions of contextual and clinical factors or rely on general frameworks, such as [33] can be applied to the creation of the cards.

3.4 Incident Card Composition

The INCIDENT CARD design encapsulates an experience in a structured format, helping researchers translate participant data into research findings and inform design. The cards underwent an iterative refinement based on feedback from two pilot studies and design iterations. An INCIDENT CARD, see Figure 1 (left), features a title, date, participant quote, representative image, and structured clinical and contextual information.

The **title (top center)** is generated by the researchers creating an INCIDENT CARD. The title can have several uses: 1) to summarize INCIDENT CARD for easy reference, 2) to encourage participants to reflect on their perspective, or 3) to draw attention to an aspect of the card that may spark meaningful discussion. The **date of the incident (top right)** helps with event recall and, in contexts where tracking applications like glucose or food monitors are used, to support cross-referencing with external data. The **participant quote (top left)** is intended to describe the incident in more detail. The quote should help participants mentally revisit and self-reflect on their interpretation of the incident. The **representative image (top right)** is a visual representation of the incident. The image supports expression, reference, inspiration, and contextualization of the incident. Images are widely used in HCI to aid self-expression [2], make abstract concepts more tangible [73], and involve participants through emotional impact [2]. Research also shows that images linked to text improve recall of health information compared to text alone [41].

Structured clinical and contextual information (table) combines two essential sources of information, clinical and contextual, into a format that supports their joint use. The table shows the contextual and clinical factor names on the left. On the right is a factor in a specific incident. For example, in Figure 1 the factor “emotional state of the participant” is manifested as annoyed, impatient, and frustrated in this incident. Contextual information can enhance recall and help participants construct associations and understand factors in their experience [45]. Supporting associations between outcome measures and contextual factors is one of the primary reflection needs of patients with chronic conditions [57]. Contextual information can enable better data analysis and interpretation [17, 75]. Figure 1 shows an INCIDENT CARD with structured clinical and conceptual information inspired by the contextual frames and factors of Raj et al. [72].

4 Incident Cards Use

To understand the role that INCIDENT CARDS might play in research activities with individuals with chronic conditions, we used them in a recent study on T1D self-management. The study included a 5-day diary study and a subsequent collaborative workshop with 10 individuals with T1D (4 were female, 6 male, 0 non-binary), age varied from 20 to 72 ($M=38.2$). In the diary study, participants collected images and voice messages related to self-management, inspired by voice-mail and elicitation diary study [15, 68]. Participants recorded on average 5.6 images and 5.5 quotes per day. The diary study transcripts and images were used to identify quotes and images for INCIDENT CARDS. For *data familiarization* we followed Braun and Clarke [9]’s approach. In *requirement definition* we selected incidents that were either unique to a participant or showed different

approaches between participants. To *map* incidents to requirements we grouped incidents based on thematic similarity, such as managing high blood glucose after a meal or strategies for physical activity. This allowed us to compare self-management strategies within similar situations and identify different approaches between participants. Our initial incident set comprised 6–10 incidents per participant, totaling 75 initial incidents. In *participant validation*, we selected four incidents per participant, though time allowed discussion of only one or two. To ensure participants’ comfort, they could choose which incidents to share, as some, while interesting, were also sensitive and personal. For INCIDENT CARD discussion, we planned to form three groups of three to four participants each, aiming for diversity in terms of personalities and discussion topics in each group. This resulted in a close connection between *final incident selection* and group allocation, mutually influencing each other.

For INCIDENT CARD composition (see Section 3.4) we opted for a title that summarizes the incident while drawing attention to a specific aspect (see Figure 1). The quote and image were based on our final incident set, and quotes were shortened when necessary. For structured contextual and clinical information, we adapted Raj et al. [72]’s contextual frames (see Section 3.3). Depending on the incident selected, not all contextual and clinical factors were derivable from the data. Upon participant feedback, we decided to encourage them to complete, adapt, and correct INCIDENT CARDS, further discussed in Section 5.3. Participants edited 34 of the 40 cards and made 56 additions and 10 corrections, showing participants’ enthusiasm to make the card feel authentic to their experience.

The workshop started with breakout groups of four to five participants and one facilitator, followed by an unsupervised plenum discussion, similar to a low-moderator involvement focus group [53]. We provided INCIDENT CARDS to participants in physical form, based on research showing that data physicalization supports reflection [82], facilitates understanding, fosters collaborative engagement, and helps participants recall activities [66]. The insights from the workshop are discussed in Section 5.

5 Incident Card Insights and Reflection

In this section, we reflect on our experiences using INCIDENT CARDS in a workshop. We discuss findings and insights of INCIDENT CARDS usage and usefulness and situate them in existing research.

5.1 Grounding Discussion and Fostering Engagement

Our workshop observations suggest that INCIDENT CARDS may help with participant engagement and grounding participant discussion in lived experiences.

Participants used INCIDENT CARDS to provide detailed accounts of specific events, rather than generalizing across experiences. This was evident in participants’ detailed descriptions of incidents during discussion, often expanding on the information on the card. Providing INCIDENT CARDS in a physical format seemed beneficial, as they were used as a reference point to clarify or expand on participants’ perspectives throughout the conversation. For example, a participant referred to his INCIDENT CARD after hearing others’ perspectives and ideas on the incident.

“I also wrote here (pointing to an INCIDENT CARD) that it’s something I need to keep learning again and again. Or I need to trust more that what I entered is actually correct. So, it’s exactly as you said - this learning to trust a bit more” (P7)

We observed that INCIDENT CARDS fostered engagement by helping participants select conversation topics and providing a clear starting point for discussion. In one case, a participant took out an INCIDENT CARD called “pizza final boss” to start the plenum discussion: “Yes, so what we discussed earlier is the pizza problem. (pointing at the incident card)” (P6). Many interview and workshop guides note the importance of an easy conversation entry to build rapport [8], break the ice [16], and prompt an opening statement or meaningful report [53]. Participant engagement is a key challenge in conducting workshops. Based on pre-workshop interactions, we observed that previously reserved participants quickly shared and contributed discussion topics, suggesting INCIDENT CARDS help mitigate the challenge of taking initiative for reserved participants [36]. If a conversation slowed down, a participant would introduce another INCIDENT CARD, keeping the dialogue active. This suggests that INCIDENT CARDS can be used as prompts in design ideation sessions, sparking creativity and engagement.

The conversations INCIDENT CARDS spurred were versatile and dynamic, as participants could switch fluidly between sharing their own experiences, related INCIDENT CARDS, or reacting to others’ perspectives. An example is that participants often asked how the others would handle the same incident.

“Yes, maybe a question at this point: how do you deal with such situations? When you know it’s correct, but you still question yourself, like I do, and start correcting things even though it’s not necessary. Do you know that feeling?” (P1)

This exchange encouraged collaborative reflection and problem-solving in participants (see Section 5.2), enabling the exploration of diverse perspectives. Our INCIDENT CARDS also seem to promote sustained engagement, by helping to transition to a new topic once one is exhausted with a new INCIDENT CARD.

5.2 Supporting Reflection

INCIDENT CARDS help participants “fill in the blanks” by fleshing out incidents and their perspectives on them. In our workshop, participants consistently contextualized incidents within the broader scope of their daily lives. This comprehensive articulation and contextualization provides researchers with detailed accounts and richer insights into user needs.

While reviewing the cards, participants linked incidents to the past and the future: they reflected on the recent events leading up to the incident in the time frame of hours or days and described their considerations to account for future outcomes. Past and Future are two design resources by Bentvelzen et al. [5] that encourage reflection. Below is an example of our participant setting the stage for an incident where they needed to raise blood glucose levels to be able to drive:

“It was like this: we had something for lunch, and in the afternoon, we visited colleagues and friends. However, my breakfast made my blood sugar rise more

than I would have liked. So, I corrected it afterward, which I then regretted in hindsight. I saw that it was high, panicked, and felt like I needed to do something immediately, so I became a bit impatient. Then we had to drive there by car. But you have to be above 5 mmol/L to drive a car.” (P7)

The participant critically reflected on the decisions that led to this incident, similar to Schön [76]’s reflection-on-action, a prominent definition of reflection used widely in HCI. Reflection-on-action happens after the event and includes thinking of what one might do differently [76], as seen in our example. This shows that INCIDENT CARDS encourage participants to reflect on past decisions and allow contextual embedding, which we might attribute to the contextual and clinical factors rubric presented on our Incident Cards. The participant’s retrospective framing highlights the importance of preceding events in understanding the incident. Future iterations on INCIDENT CARDS could support this practice with prompts like: *Think about what happened in the 12 hours before this incident.* The future perspective shows that INCIDENT CARDS encourage participants to reflect beyond the incident itself. They reflect on its long-term implications for self-management and coping strategies for similar situations.

Participants compared their INCIDENT CARD to their norm or others’ experiences. Self-comparison helped participants establish potential gaps in understanding and determine whether the incident was ordinary or extraordinary. One participant, for example, compared the experience described on the card (perceived as abnormal) to the experience on the morning of the workshop (perceived as normal), creating a gap in understanding. “So that’s how it is compared to today. I ate the exact same thing for breakfast and took the same insulin dose, but there’s just no explanation.” (P9). Establishing gaps in understanding can lead to sensemaking and essential activity for successful chronic condition management [59].

Participants also responded to others’ INCIDENT CARDS by comparing them with their own experiences. One participant shared an INCIDENT CARD on a blood glucose spike that unexpectedly returned to normal without the use of any glucose-lowering medication. Another participant then compared her experience with using a pump to find a reason for the drop in blood glucose.

“Are you also using a pump that automatically corrects your blood glucose? Cause then - I would think that this is the reason for why it came back down. But that is not the case?” (P4)

This type of collaborative problem-solving, instigated by INCIDENT CARDS, indicates their potential to drive collaborative reflection and co-design processes. We consider that co-design processes may use INCIDENT CARDS to identify challenges, and participant differences and help develop personalized solutions.

5.3 Incorporating Researchers’ Interpretations and Participants’ Experiences

During our pilot study, we observed that participants wanted to make sure that INCIDENT CARDS reflected their insights and their experiences. When we implemented this change in our workshop, participants appreciated being able to have their perspectives represented on the INCIDENT CARD. In Figure 1 (right) are the edits made

by one participant. For the blood glucose curve at the bottom, they took out their continuous glucose monitoring device, using the date on the INCIDENT CARD as an anchor point. Allowing participants to edit aligns with the HCI practice of member checking. Member checking allows participants to give feedback on transcripts for validation purposes [23, 52, 61], to correct errors [81], to eliminate the possibility of misrepresentation during the data analysis process [14]. Leaving portions of the structured clinical or contextual information intentionally blank may support this editing process. The editing process allows participants to supplement INCIDENT CARDS with personal insights and context, engage participants, and facilitate conversation.

This results in an INCIDENT CARD that combines participants' perspectives with researchers' interpretations. Participants seemed to value researchers' perspective and found the structure of the INCIDENT CARDS useful. An example of this is that participants often referred to the title of the INCIDENT CARD to frame their narratives or make statements about researchers' interpretation of the incident: "Exactly, I'm just very careful and anticipatory - that was summarized well by [author]." (P4). One participant used the title "pizza final boss" to describe self-management challenges with pizza, while another used "coffee effect" to discuss how coffee's physical effects relate to a discussion on mental stressors in self-management. This indicates that researchers' interpretation of experiences can influence how participants perceive and discuss them. The extent to which researchers want their perspectives to influence participants or activities could be something to consider when creating INCIDENT CARDS.

6 Summary and Future Work

We developed INCIDENT CARDS to help researchers and designers create an artifact that translates the experiences and practices of chronic condition management. Through a diary study, followed by a workshop, we tested our method for creating INCIDENT CARDS and explored their usefulness in externalizing experiences in a group setting. Our findings indicate that INCIDENT CARDS help ground participant discussions in their lived experiences and support participants in contextualizing their experience by fleshing out incident details and their perspectives on them. This self-reflection is enriched when using INCIDENT CARDS to compare participant practices to one another, leading to collaborative brainstorming of solutions and to new insights. Using INCIDENT CARDS, as conversation prompts, facilitates natural topic shifts while maintaining an open-ended, adaptable discussion.

To further validate and refine our INCIDENT CARDS, we plan to use them in settings beyond workshops, such as in interviews and generative design activities. A limitation is that given the workshop setting, there is no way to clearly determine if a phenomenon occurred as a result of the artifact or the activity in which it is used. Similarly, it is not possible to compare discussions with and without INCIDENT CARDS to assess whether they constrain the conversation and limit the emergence of unexpected topics. We aim to address this by evaluating the cards in a variety of other activities and comparing similar activities conducted with and without INCIDENT CARDS. We believe that INCIDENT CARDS hold potential as valuable artifacts applicable in broad contexts. Thus, we will evaluate them

in other domains to show their broader applicability to encapsulate user experiences. We will conduct several design iterations and adapt the INCIDENT CARDS to improve their usefulness with diverse domains and settings of use. Ultimately, we envision our INCIDENT CARDS and their creation to effectively encapsulate user experiences and translate them into an artifact, to the benefit of researchers and participants alike, in interviews, workshops, and design processes.

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